

=====

=====

SPRINGFIELD MEDICAL CENTER
LABORATORY REPORT

=====

=====

PATIENT INFORMATION

Name: Martinez, Robert J.
Date of Birth: 08/14/1960
Age: 65 years
Sex: Male
MRN: SMC-2019-084523
Account #: A-2026-0128-4471
Physician: Sarah Chen, MD
Location: Outpatient - Internal Medicine Clinic

SPECIMEN INFORMATION

Collection Date: 01/28/2026 07:15 AM
Received Date: 01/28/2026 07:32 AM
Report Date: 01/28/2026 02:45 PM
Specimen Type: Venous Blood (Fasting), Urine (Random)
Fasting Status: Yes (12 hours)

=====

=====

COMPLETE BLOOD COUNT (CBC)

=====

=====

Test Flag	Result	Units	Reference Range
White Blood Cell Count	6.8	x10 ³ /uL	4.0 - 11.0
Red Blood Cell Count	4.52	x10 ⁶ /uL	4.50 - 5.50
Hemoglobin	13.8	g/dL	13.5 - 17.5
Hematocrit	41.2	%	38.0 - 50.0
MCV	91.2	fL	80.0 - 100.0
MCH	30.5	pg	27.0 - 33.0
MCHC	33.5	g/dL	32.0 - 36.0
RDW	13.8	%	11.5 - 14.5
Platelet Count	218	x10 ³ /uL	150 - 400
MPV	9.8	fL	7.5 - 12.0

DIFFERENTIAL

Neutrophils	62.1	%	40.0 - 70.0
Lymphocytes	28.4	%	20.0 - 40.0
Monocytes	6.2	%	2.0 - 8.0
Eosinophils	2.8	%	1.0 - 4.0
Basophils	0.5	%	0.0 - 1.0
Neutrophils (Absolute)	4.22	x10 ³ /uL	1.5 - 8.0
Lymphocytes (Absolute)	1.93	x10 ³ /uL	1.0 - 4.0
Monocytes (Absolute)	0.42	x10 ³ /uL	0.1 - 1.0
Eosinophils (Absolute)	0.19	x10 ³ /uL	0.0 - 0.5
Basophils (Absolute)	0.03	x10 ³ /uL	0.0 - 0.2

COMPREHENSIVE METABOLIC PANEL (CMP)

Test Flag	Result	Units	Reference Range
Glucose, Fasting	145	mg/dL	70 - 100
HIGH			
Blood Urea Nitrogen (BUN)	28	mg/dL	7 - 20
HIGH			
Creatinine, Serum	1.8	mg/dL	0.7 - 1.3
HIGH			
eGFR (CKD-EPI 2021)	42	mL/min/ 1.73m ²	>= 90
LOW			
BUN/Creatinine Ratio	15.6	ratio	10 - 20
Sodium	139	mEq/L	136 - 145
Potassium	4.8	mEq/L	3.5 - 5.0
Chloride	102	mEq/L	98 - 106
Carbon Dioxide (CO ₂)	22	mEq/L	23 - 29
LOW			
Anion Gap	15	mEq/L	8 - 16
Calcium, Total	9.2	mg/dL	8.5 - 10.5
Protein, Total	6.8	g/dL	6.0 - 8.3
Albumin	3.9	g/dL	3.5 - 5.0
Globulin	2.9	g/dL	2.0 - 3.5
A/G Ratio	1.3	ratio	1.0 - 2.5
Bilirubin, Total	0.8	mg/dL	0.1 - 1.2
Alkaline Phosphatase	78	U/L	44 - 147
AST (SGOT)	24	U/L	10 - 40
ALT (SGPT)	28	U/L	7 - 56

=====

HEMOGLOBIN A1C

=====

=====

Test Flag	Result	Units	Reference Range

Hemoglobin A1c	7.2	%	4.0 – 5.6
HIGH			
Estimated Avg Glucose	160	mg/dL	(calculated)

Interpretation:
HbA1c 7.2% corresponds to an estimated average glucose of 160 mg/dL.
Target for most adults with diabetes: < 7.0% (ADA 2026).
Previous results: 6.8% (07/15/2025), 6.5% (01/20/2025).
Trend: Increasing over 12 months.

=====

LIPID PANEL (FASTING)

=====

=====

Test Flag	Result	Units	Reference Range

Total Cholesterol	178	mg/dL	< 200
Triglycerides	165	mg/dL	< 150
HIGH			
HDL Cholesterol	42	mg/dL	>= 40
LDL Cholesterol (Direct)	88	mg/dL	< 100
VLDL Cholesterol (Calc)	33	mg/dL	< 30
HIGH			
Non-HDL Cholesterol	136	mg/dL	< 130
HIGH			
Total Chol/HDL Ratio	4.2	ratio	< 5.0

=====

RENAL PANEL (ADDITIONAL)

=====

=====

Test Flag	Result	Units	Reference Range

Phosphorus, Serum	4.2	mg/dL	2.5 – 4.5
Uric Acid, Serum	7.8	mg/dL	3.5 – 7.2
HIGH			
Magnesium, Serum	1.9	mg/dL	1.7 – 2.2
Intact PTH	68	pg/mL	15 – 65
HIGH			

URINE STUDIES

Test Flag	Result	Units	Reference Range
Urine Albumin HIGH	28.8	mg/L	< 20
Urine Creatinine	160	mg/dL	(varies)
Urine Albumin/Creatinine HIGH Ratio (UACR)	180	mg/g	< 30

Interpretation:

UACR 180 mg/g indicates moderately increased albuminuria (Category A2).

Categories: A1 (< 30 mg/g), A2 (30–300 mg/g), A3 (> 300 mg/g).

Previous results: 145 mg/g (07/15/2025), 95 mg/g (01/20/2025).

Trend: Increasing. Consistent with diabetic nephropathy.

COMMENTS

- CKD staging based on eGFR 42 mL/min/1.73m² and UACR 180 mg/g:
CKD Stage G3a–A2 (Moderately decreased GFR, moderately increased albuminuria). KDIGO risk category: HIGH.
- Declining eGFR trend noted:
 - 01/20/2025: eGFR 55 mL/min/1.73m²
 - 07/15/2025: eGFR 48 mL/min/1.73m²
 - 01/28/2026: eGFR 42 mL/min/1.73m²
 Rate of decline: approximately 13 mL/min/year. Exceeds threshold of > 5 mL/min/year -- nephrology referral recommended if not already in place.
- Mildly low CO₂ (22 mEq/L) may indicate early metabolic acidosis

associated with CKD. Consider serum bicarbonate monitoring and supplementation if confirmed < 22 mEq/L on repeat testing.

4. Elevated PTH (68 pg/mL) is consistent with early secondary hyperparathyroidism of CKD. Monitor calcium, phosphorus, and PTH per KDIGO guidelines.
5. Elevated uric acid (7.8 mg/dL) is common in CKD. Monitor; consider treatment if symptomatic (gout) or if further renal decline.

=====

PERFORMING LABORATORY

=====

Springfield Medical Center Clinical Laboratory
100 Medical Center Drive, Springfield, IL 62704
CLIA #: 14D0987654

Laboratory Director: Michael R. Thompson, MD, PhD
Report generated electronically. Verified by: Jennifer A. Kowalski,
MT(ASCP)

=====

*** END OF REPORT ***

=====